

SUNRISE HOME HEALTH CARE

Employee Policy & Operational Handbook

A Comprehensive Guide to Regulatory Standards, Mobile Care Eligibility, Clinical Documentation, and Corporate Compliance Frameworks.

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Applicability:	All Full-Time, Part-Time, and Contracted Clinical Field Staff

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1. REMOTE WORK & MOBILE CARE ELIGIBILITY

Sunrise Home Health Care leverages a decentralized operational model to deliver clinical excellence directly into patients' residences. This section governs the qualifications, structural requirements, and safety guidelines that define parameters for mobile clinical operations and remote administrative tasks.

1.1 Clinical Field Classification & Minimum Requirements

Field employees include Registered Nurses (RNs), Licensed Practical Nurses (LPNs), Physical Therapists (PTs), Occupational Therapists (OTs), Speech-Language Pathologists (SLPs), and Certified Home Health Aides (CHHAs). To maintain structural eligibility for independent field placement, clinical personnel must strictly satisfy the following operational baselines:

- **Licensure and Credentials:** Hold and maintain an active, unencumbered professional license in the state of practice. Verification must be uploaded to the primary HR portal semi-annually.
- **Competency Assurances:** Complete 40 hours of initial onboarding field training and pass the Sunrise Clinical Competency Assessment (SCCA) with a score of 90% or higher.
- **Health and Clearance Screenings:** Provide annual evidence of a negative Tuberculosis (TB) screening, updated immunization records, and pass an initial and random background check and multi-panel drug screen.

1.2 Workspace Safety & Home Office Standards

When clinical personnel perform administrative tasks, care coordination, or remote telehealth tracking from their residential premises, the designated remote environment must consistently meet standard professional safety parameters.

The home office environment must feature a physical layout that completely isolates business operations from domestic shared spaces. This isolation is mandatory to prevent accidental visual or auditory exposure to sensitive client files or active phone conversations. Personnel must maintain ergonomic seating layouts and a pathway free from physical tripping hazards.

Technologically, the designated workspace must maintain an active high-speed broadband network interface delivering minimum data synchronization metrics of 25 Mbps download and 10 Mbps upload. This performance threshold ensures continuous, uncorrupted connectivity with the enterprise Electronic Health Record (EHR) server architecture. Sunrise Home Health Care retains the right to audit these environments via mandatory virtual safety checks.

1.3 Geographical Service Boundaries & Travel Protocols

Mobile clinicians are assigned to clear regional boundaries designed to optimize care delivery density and control operational exhaustion. Field clinicians are expected to accept patient assignments within a fixed 45-mile radius of their designated regional administrative hub.

- **Compensable Travel Intervals:** Active travel between consecutive patient appointments within a single shift is classified as standard working time. The initial commute to the first patient and the return journey from the final client are explicitly classified as non-compensable personal commuting time.
- **Automobile Insurance Mandates:** All field staff utilizing personal vehicles for company deployment must maintain a valid driver's license and carry active personal automobile liability policy caps of at least \$100,000 per individual and \$300,000 per accident.

- **Extreme Weather Frameworks:** In scenarios involving hazardous weather conditions, field personnel must consult the active Inclement Weather Matrix to coordinate route modifications, ensuring patient safety remains optimized without risking personnel well-being.

2. DATA SECURITY & HIPAA COMPLIANCE

As a federally regulated covered entity, Sunrise Home Health Care strictly enforces all provisions of the Health Insurance Portability and Accountability Act (HIPAA) and the Health Information Technology for Economic and Clinical Health (HITECH) Act. Protecting client data is a core operational requirement.

2.1 Protocols for Handling Protected Health Information (PHI)

Protected Health Information (PHI) encompasses any information that can link clinical realities to a specific individual's identity. When operating within mobile environments, safeguarding physical and electronic PHI demands aggressive compliance workflows:

- **Environment Management:** Clinical records or active EHR tablet layouts must never be left unattended inside a patient's home or visible to family members or external observers.
- **Auditory Privacy Controls:** All verbal clinical interactions, clinical updates, or supervisor consultations performed outside office walls must occur in private spaces where eavesdropping risks are completely eliminated.
- **Physical Document Transport:** Physical documentation must be kept to an absolute minimum. When physical paperwork is required, it must be contained in a heavy-duty, double-locked corporate briefcase stored completely out of public view in the vehicle's locked trunk.
- **Document Destruction:** Under no circumstances may documents containing client data be discarded in public or domestic waste bins. All documentation must be brought back to a regional facility for immediate cross-cut shredding.

2.2 Mobile Device Security & Bring Your Own Device (BYOD) Policy

Mobile devices serve as the main interface for data collection and processing across the enterprise. Any device used to view or log information must be enrolled in the enterprise Mobile Device Management (MDM) software platform.

The enterprise MDM application enforces strict configurations, including a required 8-digit alphanumeric device pin, automatic screen locks that trigger after 120 seconds of inactivity, and full-disk cryptographic encryption.

Capturing digital photographs of wounds, clinical sites, or physical anatomical features must occur exclusively via the secure camera module embedded within the sandboxed corporate EHR environment. These clinical images must never be captured with a device's stock camera application, saved onto local storage, or shared via standard SMS, text message, or consumer communication applications.

2.3 Multi-Factor Authentication & Emergency Breach Protocols

System authorization requires Multi-Factor Authentication (MFA) across all digital assets, corporate portals, email accounts, and scheduling engines. Network credentials, security keys, or biometric configurations must never be shared under any circumstances. Passwords must be updated every ninety (90) days following a complex composition rule matrix.

CRITICAL TIMELINE: SYSTEM THREAT AND DATA LOSS ESCALATION

If an authorized corporate tablet, personal BYOD cell phone, or physical encrypted briefcase is lost, stolen, or suspected of being compromised, the employee is required to file a report with the IT Security Helpdesk within two (2) hours of discovery. Sunrise IT will execute a remote command to wipe the device and freeze all data credentials immediately.

3. TIMEKEEPING, BILLING & PAYMENT INTEGRITY

Sunrise Home Health Care operates under rigid state and federal healthcare funding frameworks. Absolute accuracy in documenting working hours and billing metrics is a strict statutory requirement under the federal False Claims Act.

3.1 Electronic Visit Verification (EVV) Compliance

Pursuant to mandates defined by the 21st Century Cures Act, all in-home personal care and clinical services provided by Sunrise Home Health Care must be validated using our automated Electronic Visit Verification (EVV) system.

- **Point-of-Care Validation:** Field personnel must initiate an active check-in cycle on the Sunrise Care Mobile Application at the exact moment they arrive at the patient's physical bedside and complete a check-out log immediately when the care session ends.
- **Data Points Captured:** The EVV infrastructure automatically records precise GPS coordinates, cryptographically signs timestamps, tracks user ID parameters, and documents client confirmation inputs.
- **Manual Entry Mitigation:** Retrospective edits or manual time modifications are highly restricted. If a technical failure occurs, a manual log adjustment request must be filed within 48 hours, supported by a signed patient clinical validation form and explicit approval from the Regional Clinical Director.

3.2 Real-Time Clinical Documentation & Fraud Prevention

Clinical progress logs, vital monitoring details, and medication records must be logged directly into the system within 24 hours of the clinical visit. Delays compromise patient safety and violate standard billing regulations.

The practice of clinical documentation cloning—defined as copying narrative text blocks across different dates or distinct patients—is strictly prohibited. All documentation must reflect a unique assessment of the client's current status. Upcoding, billing for care not rendered, documenting visits that were canceled, or expanding billing intervals beyond face-to-face care hours constitutes illegal activity subject to criminal prosecution.

3.3 Overtime Pre-Approval & Corporate Auditing Systems

Field staff must complete their assignments within their authorized weekly shift caps. Working overtime requires explicit written pre-approval from the immediate Clinical Supervisor, except during unexpected acute emergency stabilization tasks.

Travel reimbursements are computed using automated GPS distance tracking metrics within the mobile platform. Manual mileage exception requests must be backed by a detailed route manifest. The Corporate Compliance Office runs monthly random audits on 10% of all submitted timesheets, cross-referencing mobile GPS pings against clinical chart details to identify anomalies.

4. INCIDENT REPORTING & DISCIPLINARY ACTIONS

Continuous quality improvement and maximum patient safety require prompt visibility into unexpected errors, physical incidents, or procedural failures. This section sets forth the reporting expectations and accountability frameworks across our operations.

4.1 Classification of Critical Safety Incidents

An incident is defined as any unplanned occurrence that impacts, or poses an imminent threat to, the health, safety, or legal rights of a Sunrise patient or employee. Critical incidents are categorized as follows:

- **Medication Errors:** Omitting a medication dose, administering an incorrect drug, delivering an inaccurate dose, or executing administration at an unauthorized time.
- **Patient Injury Events:** Falls, cuts, burns, or physical injuries sustained during transfers or mobility support within the residential environment.
- **Abuse, Neglect, or Exploitation:** Any situation where an employee witnesses or suspects emotional, physical, financial, or clinical abuse of a vulnerable patient by anyone. Under mandatory reporting laws, clinicians must immediately report these findings to Adult Protective Services.

4.2 Mandatory Reporting Timelines & Whistleblower Protections

When a critical incident occurs, field staff must first ensure the immediate safety and stabilization of the patient, contacting emergency medical services (911) if clinically indicated. A verbal alert must be delivered to the Clinical Supervisor within one (1) hour of the event.

Subsequently, a comprehensive, formal online Incident Report Form (IRF) must be completely filled out and submitted through the internal compliance link within twelve (12) hours of discovery.

Sunrise Home Health Care enforces a strict non-retaliation environment for employees. Whistleblower protections ensure that any employee who reports systemic clinical non-compliance, financial fraud, or patient safety hazards in good faith is completely protected against any form of professional reprisal, discrimination, or career penalty.

4.3 Progressive Disciplinary Action Framework

To preserve administrative integrity and safety, the company utilizes a structured progressive discipline framework designed to correct behavioral errors. The standard progression consists of:

- **Level 1: Documented Verbal Counseling.** A documented meeting defining specific operational gaps and establishing a clear 30-day monitoring timeline.
- **Level 2: Formal Written Warning.** A formal document placed in the employee's permanent personnel file, outlining the specific policy violation and a mandatory performance plan.
- **Level 3: Privilege Suspension & Mandatory Retraining.** Temporary removal from the field clinical rotation to undergo mandatory targeted educational modules.
- **Level 4: Administrative Separation.** Permanent termination of employment with Sunrise Home Health Care.

The company reserves the absolute right to bypass progressive steps and proceed directly to immediate termination for zero-tolerance behaviors. These behaviors include: intentional theft or unauthorized distribution of patient PHI, controlled substance diversion or theft, any form of physical or verbal client abuse, and deliberate falsification of clinical documentation or corporate timekeeping records.